

1. Administrative & Study Identification

Full Study Title: An Open-label Phase 1 Study to Evaluate the Safety of PF-08046045/SGN-35T in Adults With Advanced Malignancies **Short Title/Acronym:** A Safety Study of PF-08046045/SGN-35T in Adults With Advanced Cancers (SGN35T-001) **Protocol Number:** SGN35T-001 **NCT Number:** NCT06120504 **Sponsor Name:** Seagen Inc. (a wholly owned subsidiary of Pfizer) **Investigational Product:** PF-08046045 / SGN-35T (CD30-directed antibody-drug conjugate) **Phase of Development:** Phase 1 **Medical Monitor:** Tara Chen (clinicaltrials@seagen.com)

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the safety, tolerability, and identify the recommended dose and schedule of PF-08046045/SGN-35T in adult patients with relapsed/refractory (R/R) CD30-expressing lymphoid malignancies. **Secondary Objectives:** To evaluate pharmacokinetics (PK), pharmacodynamics, objective response rate (ORR), duration of response (DoR), complete response (CR) rate, and the incidence of antidrug antibodies.

2.2 Background & Rationale Patients with relapsed/refractory (R/R) lymphomas have limited treatment options and poor mortality rates. CD30 is a validated therapeutic target in R/R lymphoid malignancies. Brentuximab vedotin (BV), a CD30-directed antibody-drug conjugate (ADC), has demonstrated clinical benefit but is associated with tolerability limitations. SGN-35T is an investigational ADC comprised of the same anti-CD30 monoclonal antibody backbone as BV, but it is conjugated to monomethyl auristatin E (MMAE) via a novel protease-cleavable tripeptide linker. This novel linker is designed to preferentially release MMAE directly inside target cells to improve tolerability and efficacy through direct cytotoxicity, CD30+ regulatory T-cell depletion, and immunogenic cell death.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Single-arm (Dose-escalation and dose-expansion) **Blinding:** Open-label **Randomization:** N/A

3.2 Planned Enrollment & Duration Target \$N\$: To be determined based on dose escalation parameters (Phase 1 study format) **Number of Sites:** Multicenter, Global (including the US and Spain)

Estimated Study Start: Late 2023 / Early 2024 **Estimated Primary Completion:** Not explicitly stated (Ongoing / Recruiting)

4. Subject Selection (Eligibility Criteria)

- 4.1 Inclusion Criteria** 1. Men and women ≥ 18 years of age with an Eastern Cooperative Oncology Group (ECOG) performance status ≤ 1 . 2. Histologically confirmed R/R lymphoid malignancy (including classical Hodgkin lymphoma, peripheral T-cell lymphoma, mature B-cell lymphoma, or primary cutaneous lymphoma) with no standard therapy available. 3. Detectable CD30 expression ($\geq 1\%$) in tumor tissue from the most recent biopsy, except in subtypes where CD30 is universally expressed (e.g., cHL and sALCL).
- 4.2 Exclusion Criteria** 1. Prior treatment with more than 2 brentuximab vedotin-based lines of therapy. 2. History of another malignancy within 3 years before the first dose of study intervention. 3. Active cerebral/meningeal disease related to the underlying malignancy. 4. Received previous autologous stem cell transplant (ASCT) < 12 weeks prior to the first dose, or allogeneic stem cell transplant (SCT) < 100 days prior, or active graft-versus-host disease. 5. Grade ≥ 2 pulmonary disease unrelated to underlying malignancy, or clinically significant lung disease requiring systemic corticosteroids within 6 months prior to enrollment.
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5. Intervention & Treatment Plan

- 5.1 Investigational Regimen Dosage/Frequency:** PF-08046045/SGN-35T administered over a 3-week period (21-day cycle). Parts A and B evaluate dose-escalation and optimization; Part C and the biology cohort receive the recommended phase 2 dose. **Route of Administration:** Intravenous (IV) infusion. **Duration of Treatment:** Administered in 3-week cycles until disease progression, unacceptable toxicity, or study withdrawal.
- 5.2 Concomitant & Prohibited Therapy Permitted:** Standard supportive care for managing disease symptoms. **Prohibited:** Concurrent investigational agents, immunosuppressive therapy for graft-versus-host disease, and systemic corticosteroids used for lung disease within 6 months of enrollment.
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6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures	:---
:---	:---	Screening	Day -28 to 0 Informed Consent, Medical History, Tumor Biopsy (CD30 testing), Baseline PET, MRI, or CT Scan
Baseline	Day 1	First IV Infusion of PF-08046045/SGN-35T	Interim End of Cycle 2 & Q3 Months

Safety Labs, PK/PD Assessments, Tumor Assessment Scans (PET/CT/MRI) every 3 months after Cycle 2 | | **End of Study** | Variable | Final Tumor/Safety Assessment, Survival Follow-Up |

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Incidence and severity of adverse events (AEs) and laboratory abnormalities, frequency of dose modifications, and incidence of dose-limiting toxicities (DLTs). **Measurement Method:** Clinical observations, physical examinations, and standard laboratory testing per the NCI CTCAE.

7.2 Secondary & Exploratory Endpoints * Pharmacokinetics (PK) parameters and incidence of antidrug antibodies. * Objective Response Rate (ORR) and Complete Response (CR) rate. * Duration of Response (DoR).

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous evaluation for AEs and DLTs to identify the optimal dosing schedule. Specific monitoring for pulmonary symptoms and interstitial lung disease (ILD).

Serious Adverse Events (SAEs): Reported per standard clinical trial regulatory timelines (typically 24 hours). **Data Monitoring Committee (DMC):** Internal safety review committee utilized for dose-escalation decisions between study cohorts.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Safety Analysis Set (all patients receiving at least one dose of the investigational product). **Sample Size Justification:** Sample size is determined by standard Phase 1 dose-escalation algorithms (e.g., 3+3 or continuous reassessment methods) aiming to identify the Maximum Tolerated Dose (MTD) or Recommended Phase 2 Dose. **Handling of Missing Data:** Missing data will be handled using descriptive statistics without imputation for safety assessments, per standard early-phase oncological trial protocols.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Regular monitoring of clinical sites to ensure adherence to the dose-escalation protocol, safety reporting, and PK/PD sample handling. **Audit Trail:** Secure eCRF locking and compliance checks for tumor response imaging and biopsy handling.

11. Study Identifiers & Governance

Primary ID: {{SGN35T-001}} Registry Number: {{NCT06120504}}
Sponsor/Lead Organization: {{Seagen Inc. (Pfizer)}} Study Title: {{A Safety Study of PF-08046045/SGN-35T in Adults With Advanced Cancers}} Official Regulatory Title: {{An Open-label Phase 1 Study to Evaluate the Safety of PF-08046045/SGN-35T in Adults With Advanced Malignancies}}

12. Clinical Framework (Lymphoma Specific)

Primary Condition: {{Relapsed/Refractory Lymphoma (cHL, PTCL, sALCL, B-cell, PCL)}} Diagnosis Threshold: {{CD30 expression \$\ge 1\%\$}} Medical Methodology: {{Targeted Antibody-Drug Conjugate (ADC) Therapy}} Optimization Target: {{Tumor response rate and toxicity minimization}}

13. Study Procedures & Methodology

Management Pathway: {{Dose-escalation (Parts A/B) followed by dose-expansion (Part C)}} Education Protocol (HCP): {{Site initiation and dose-limiting toxicity (DLT) training}} Education Protocol (Patient): {{Informed consent, infusion expectations, and AE reporting}} Quality Audit Mechanism: {{Central review of imaging scans and centralized PK/PD laboratory analysis}}

14. Observation & Follow-Up Schedule

Total Duration: {{Treatment until disease progression or unacceptable toxicity}} Onsite Visit Cadence: {{Every 3-week cycle for infusion}} Remote Monitoring Frequency: {{As needed between cycles}} Checklist Review Interval: {{Imaging scans performed at end of Cycle 2, then every 3 months}}

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				